VA Telehealth Experience Modernization: 2025-2026 Strategic Brief

Classification: Internal Use Only **Prepared by:** VA Office of Connected Care, Digital Experience Team **Date:** October 2024 **Version:** 2.1

Purpose

This brief outlines the strategic direction for modernizing the Veteran telehealth experience, with specific focus on the appointment joining and onboarding flows. It synthesizes findings from recent research, stakeholder input, and technical assessments to inform the Telehealth Clinic Redesign initiative launching in early 2026.

Current State Assessment

By the Numbers

Metric	2023	2024 (Projected)	2026 Target
Total VVC sessions	3.1M	4.2M	6.0M
First-attempt join success	66%	68%	90%
Veteran satisfaction (CSAT)	72%	74%	85%
Technical support calls	890K	1.1M	400K
Average time to join	4.8 min	4.5 min	1.5 min

Pain Point Summary

Based on aggregated research from OIG reports, user research studies, and contact center analysis:

Top 5 Veteran Pain Points:

1. **Confusing multi-channel notifications** (38% of complaints)
2. **Browser/device permission issues** (24% of complaints)
3. **No clear path when something goes wrong** (18% of complaints)
4. **Forgetting appointments or joining late** (12% of complaints)
5. **Accessibility barriers for AT users** (8% of complaints)

Top 5 Provider Pain Points:

1. Waiting for Veterans who can't connect (avg. 7 min wait)
2. Spending clinical time on technical support

3. Inconsistent audio/video quality affecting care
4. Limited ability to help Veterans troubleshoot remotely
5. No visibility into Veteran's technical readiness

Strategic Priorities

Priority 1: Simplified Joining Experience

Goal: Reduce time-to-join from 4.5 minutes to under 1.5 minutes

Initiatives:

- One-click join from unified notification
- Automatic device detection and configuration
- Eliminate unnecessary confirmation steps
- Pre-loaded credentials from VA.gov authentication

Success Metrics:

- First-attempt success rate: 66% → 90%
- Average time to join: 4.5 min → 1.5 min
- Technical support call volume: -50%

Priority 2: Proactive Technical Readiness

Goal: Identify and resolve technical issues before appointment time

Initiatives:

- Automated device/connection test 24 hours before appointment
- Proactive SMS/email with test results and resolution steps
- "Green light" indicator visible to schedulers and providers
- Fallback options (phone, reschedule) presented automatically when issues detected

Success Metrics:

- Technical issues resolved pre-appointment: 70%
- No-show rate due to technical issues: -40%
- Provider wait time for Veteran connection: -60%

Priority 3: Accessible-First Design

Goal: Full WCAG 2.1 AA compliance with enhanced support for common Veteran accessibility needs

Initiatives:

- Screen reader optimization for VVC interface
- Improved caption accuracy (71% → 95%)
- High-contrast and large-text modes
- Cognitive accessibility features (simplified mode, step-by-step guidance)
- Caregiver co-pilot features

Success Metrics:

- Accessibility audit score: 95%+
- AT user satisfaction: 60% → 80%
- Caregiver-assisted session success rate: 85% → 95%

Priority 4: Intelligent Support Escalation

Goal: Provide the right level of support at the right time

Initiatives:

- Contextual troubleshooting based on detected issues
- One-click escalation to human support with full context transfer
- Virtual assistant for common questions (with easy human handoff)
- Provider-initiated technical help for struggling Veterans

Success Metrics:

- Self-service resolution rate: 60%
- Time to human support when needed: <30 seconds
- Support interaction satisfaction: 80%+

Competitive Landscape

How VA Compares

Feature	VA VVC	Teladoc	Amwell	Doxy.me
No download required	✅	❌	❌	✅
One-click join	❌	✅	✅	✅
Pre-appointment tech check	❌	✅	❌	❌
Integrated chat support	❌	✅	✅	❌

Feature	VA VVC	Teladoc	Amwell	Doxy.me
Caregiver features	❌	❌	❌	❌
Accessibility (WCAG AA)	Partial	Partial	Partial	❌

Competitive Insight: No major telehealth platform currently excels at accessibility or caregiver support—this represents a differentiation opportunity for VA.

Technical Architecture Considerations

Current State

- VVC is a separate application from VA.gov and MyHealtheVet
- Authentication requires separate login flow
- Notifications sent through multiple, uncoordinated systems
- Limited data sharing between scheduling and VVC systems

Target State (2026)

- Unified experience within VA.gov health portal
- Single sign-on with VA.gov credentials
- Consolidated notification system with Veteran preferences
- Real-time technical readiness data available to scheduling and clinical systems

Key Dependencies

- VA.gov platform modernization timeline
- Identity verification system upgrades (Login.gov integration)
- Notification platform consolidation (VA Notify)
- EHR integration for scheduling data

Stakeholder Alignment

Executive Sponsors

- Under Secretary for Health — Overall accountability
- CTO, Office of Information & Technology — Technical delivery
- Chief Veterans Experience Officer — Veteran satisfaction outcomes

Key Partners

- Office of Connected Care — VVC platform ownership
- VA.gov Platform Team — Integration and hosting
- Contact Center Operations — Support experience
- Office of Accessibility — Compliance and testing
- Veterans Service Organizations — Veteran advocacy and feedback

Governance

- Monthly steering committee reviews
- Quarterly OKR assessments
- Continuous Veteran feedback integration through medallia and research

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Investment Requirements

FY2025 Budget Request: $12.4M

Category	Amount	Purpose
Development	$6.2M	Platform modernization, new features
Research	$1.8M	Ongoing user research, usability testing
Accessibility	$1.5M	WCAG compliance, AT testing
Infrastructure	$1.9M	Scalability, reliability improvements
Support Operations	$1.0M	Enhanced support staffing, training

ROI Projection

- Reduced support costs: $4.2M annually (at target state)
- Avoided no-show costs: $8.1M annually
- Increased telehealth capacity: +25% without additional clinical staff
- Payback period: 18 months

Timeline

Q1 2025: Research and design validation
Q2 2025: Technical architecture finalization
Q3 2025: Development sprint 1 (joining flow)
Q4 2025: Development sprint 2 (proactive readiness)

Q1 2026: Accessibility enhancements, pilot launch
Q2 2026: Full rollout, optimization

Risks and Mitigations

Risk	Likelihood	Impact	Mitigation
Platform dependencies delayed	Medium	High	Identify standalone improvements that don't require VA.gov integration
Veteran adoption resistance	Low	Medium	Extensive change management, maintain legacy options during transition
Budget constraints	Medium	High	Prioritize highest-impact initiatives, phase implementation
Technical complexity	Medium	Medium	Incremental releases, comprehensive testing

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Next Steps

1. **November 2024:** Finalize research synthesis and design principles
2. **December 2024:** Complete technical architecture review
3. **January 2025:** Begin design sprints for joining flow
4. **February 2025:** Veteran co-design sessions
5. **March 2025:** Development kickoff

Document Owner: Digital Experience Team, Office of Connected Care **Review Cycle:** Monthly **Next Review:** November 15, 2024